EasyRS Preview — User Guide

Who this is for: invited evaluators at Canadian dental practices and orthodontic laboratories. No prior EasyRS experience needed — follow the workflows top to bottom.

Where: `https://easyrx.manqiaotechnology.com` **Accounts:** you received one practice account and one lab account by invitation email. There is no public sign-up.

> **Demo Environment — Synthetic Data Only.** Every patient, scan, and order in this environment is synthetic. Do **not** upload real patient records, identifiable scans, or clinical data — the upload form will remind you each time.

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0. At a glance — what the platform does

EasyRS runs the complete digital prescription workflow between a dental practice and its lab:

```
 Practice                              Lab
————————                              ———
Patient chart
  → Treatment Case
    → Digital Scans (STL)
      → Prescription (Rx) ————————→ Inbox
        sign & submit                → Download & Check-in
                                       → In Progress
                                       → Quality Check
                                       → Ship  ————————————→ Receive
                                                              → Delivered
```

Every step is recorded in an audit trail both sides can see. Files travel as signed, expiring links — neither side emails STL files around.

Roles in this demo

Account	Sees	Can do
`preview.doctor@demo.easyrs`	Practice nav: Patients, Prescriptions, Inbox, Settings	patients, cases, scans, Rx draft/sign/submit, comments, receive, deliver
`preview.lab@demo.easyrs`	Lab nav: Inbox, Prescriptions, Settings	download bundle, check in, complete, QC, ship, comment

Your two accounts belong to **separate demo organizations** — the practice `Harbourview Orthodontics (demo)` and the lab `Coastal Ortho Lab (demo)`. They can only see orders shared between them; nothing else in the system is visible to either account.

Conventions used in the UI

- **Upper arch / Lower arch** — which jaw a scan or appliance targets.
- **FDI tooth numbers** — e.g. tooth 14 is upper-right first premolar.
- **Dates** render as `Oct 25, 2026`; times follow your organization timezone (Settings).
- **Statuses are plain English**: Saved → Submitted → Checked-In → In Progress → Completed → QC → Shipped → Received → Delivered.
- The amber **Demo Environment** banner and the version tag stay on screen after sign-in — that is intentional.

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1. Sign in and pick your starting point

1. Open `https://easyrx.manqiaotechnology.com`.
2. The welcome card offers three real entry points — they are not decorative; each one remembers where you wanted to go:

Button	After sign-in you land on
Try the Practice Workflow	Patients (start §2)
Explore the Lab Workflow	Prescriptions inbox (start §3)
View a Sample Prescription	Prescriptions list — open the order marked Completed/Delivered (§4)

3. Enter the invited email + password → **Sign in**.
4. To switch roles, use **Sign out** (top-right) and sign in with the other account. Deep links survive sign-out/sign-in, and you can refresh on any page without losing your place.

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2. Workflow A — Practice side: patient → case → scan → prescription

2.1 Create a patient

1. **Patients** → **New patient**.
2. Fill **First name**, **Last name**, **Date of birth**; **External ref** is your own chart number (optional).
3. **Create patient** → you land on the patient chart. If the name+birthday matches an existing patient you get a confirmation prompt instead of a silent duplicate.

4. **Search** on the Patients list filters by name or ref — useful once the list grows.

The chart shows: demographics, **Treatment Cases**, **Digital Scans**, and existing prescriptions for this patient.

2.2 Open a treatment case

1. On the patient chart → **New case label** (e.g. `Hawley retainer — upper`) → **Add case**.
2. Add **Notes** — clinical context the lab should see. **Save**.

A case is the clinical folder: scans and prescriptions attach to it, so one patient can hold several independent cases.

2.3 Upload digital scans (STL)

1. On the patient chart, in **Digital Scans**: choose **STL file**, pick the **Jaw** (`upper` / `lower`), then **Upload STL**.
2. Watch the notice: *"Do not upload real patient records, identifiable scans, or clinical data."* — synthetic STL files only.
3. After upload the row shows the file size, **SHA-256** fingerprint and `valid mesh` when the file parses. Files upload via a signed URL straight to object storage — the progress bar is the real transfer, and a 200 MB+ file is rejected with a readable message.
4. Per file: **Preview** opens the 3D viewer (drag to orbit, scroll to zoom — verify the mesh actually loads), **Download** gets the original bytes back, **Hide** removes it from the case view without deleting.

Upload **both** an upper and a lower STL — the lab bundle and the Rx will reference them.

2.4 Write the prescription

1. From the patient chart (or **Prescriptions** → **New Rx**): pick the **patient** (type to filter), then the **treatment case** — the Rx is bound to the case so the lab sees its scans.
2. Pick a **Template** if your org saved one, or build items manually.
3. **Add item**: choose a catalog entry (e.g. `Hawley retainer`), the **arch** it applies to, and any required options — tooth-numbered options like `band_tooth` ask for an FDI number. Repeat for each appliance. Remove a line with **remove**.
4. Set the **Due date**, then **Save draft** — the Rx now exists as `Saved`, editable and visible only inside your practice.
5. Optional: **Template name** + save to reuse this item set later.

2.5 Sign and submit

1. Open the saved Rx. Review items, case, due date; attach anything else under **Attachments** (**Patient file** or pick an existing case scan and an **Attachment role**).
2. **Type full name to sign** — this is the clinical signature gate; the button stays disabled until the name matches.

3. **Sign & submit** → status `Submitted`. The Rx leaves "draft" and appears in the lab's inbox; edits are locked.
4. Made a mistake? Before the lab checks it in, **Return to draft** pulls it back to `Saved`. After check-in the order is owned by the lab's queue and unsubmit is refused — call them via a comment instead.

The detail page keeps a live **history** line — `Saved → Submitted` and who did it — so both sides see the same truth.

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3. Workflow B — Lab side: inbox → check-in → QC → ship

Sign in as `preview.lab@demo.easyrs`. The lab lands on Prescriptions — submitted orders are the work queue.

3.1 Accept the order (Download & Check-in)

1. Open the `Submitted` Rx.
2. **Download bundle** grabs the ZIP the practice sent: the Rx PDF, a manifest, and every attached STL. Check the contents — the manifest lists what should be inside.
3. **Download & Check-in** does both in one step and moves the order to `Checked-In` — this is the lab's formal acceptance. The practice sees `Submitted → Checked-In` in its history immediately.
4. If the files are wrong (missing arch, corrupted mesh), reject the work *now* with a comment instead of checking in.

3.2 Produce and complete

1. Work the case (outside the system in this demo).
2. **Mark complete** → `Completed`. That means production finished; quality review happens next, before anything ships.

3.3 Quality Check

Two outcomes, both recorded:

- **QC pass** → recorded in history, order becomes shippable.
- **QC fail** → requires an **Issue kind/subject/body** — e.g. *fit / clasp fracture / re-scan needed*. A fail sends the order back; a fix later re-runs the cycle.

3.4 Ship

1. **Carrier** + **Tracking number** (required pair) → **Ship**.
2. Status `Shipped`; the tracking data shows on the practice side — the same row, no email needed.

3.5 Comments & approvals (both directions, any time)

- **Add comment** on the Rx thread — visible to both orgs, each entry is attributed and timestamped.
- **Sent to counterparty inbox** / **Approval required** messages raise a notification on the other side's **Inbox** — filterable by kind (Approval required / Comment / Mention) and read state. The nav badge counts unread.

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4. Workflow C — Practice fulfillment: receive → deliver → audit

Back on the practice account:

1. Open the shipped Rx → **Confirm receipt** → `Received` (the lab sees it instantly — no "did it arrive?" phone call).
2. Fit the appliance to the patient → **Mark delivered** → `Delivered`. The lifecycle is closed.
3. Scroll the **History** — every hop is there: created, submitted, checked-in, completed, QC, shipped, received, delivered, with actor and timestamp. This is the audit trail used for disputes and records.
4. **View a Sample Prescription** shortcut takes you straight to a seeded order that already completed this loop — fastest way to see a full history without doing the whole dance.

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5. Everything else on the nav bar

Area	What it does
Patients	List/search/create charts; duplicate-name guard
Prescriptions	All Rx for your org — drafts, in-flight, history; the lab's view is its work queue
Inbox	Cross-org notifications: approvals requested, comments, mentions; kind + read filters, unread badge in nav
Settings	**Timezone** (dates everywhere follow it), default doctor/location, **locations** (add/list), **roster** (staff entries), **lab connections** (send/accept/reject/withdraw a practice↔lab link — orders only flow between connected orgs), **change password**
Sign out	Ends the session; deep links still work after re-login

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6. What is enforced for you (no configuration needed)

- **Tenant isolation** — the two demo orgs cannot see each other's patients, only the orders they share.
- **Signed files** — uploads/downloads use expiring SigV4 URLs against object storage, not emailed attachments.

- **Integrity** — every stored file carries a SHA-256; a truncated or corrupt STL is marked, not silently accepted.
- **Audit** — every state change is attributed and immutable.
- **Safety rails** — oversize files rejected with readable errors, double-submits are idempotent, destructive states are gated (sign to submit, check-in to lock, receipt to close).
- **Privacy posture** — `noindex` everywhere, secure cookies, synthetic data only, and demo data can be reset between evaluation rounds.

7. Feedback

Use the in-product thread or reply to your invitation — tell us which step felt wrong, not just what broke. Do not include real patient information in feedback.